

Methylergonovine (methylergometrine): Drug information - UpToDate

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For additional information see "Methylergonovine (methylergometrine): Patient drug information"

For abbreviations, symbols, and age group definitions show table

Brand Names: US

- Methergine

Pharmacologic Category

- Ergot Derivative

Dosing: Adult

Postpartum hemorrhage

Postpartum hemorrhage:

Oral: 0.2 mg 3 to 4 times daily in the puerperium (maximum duration: 1 week).

IM, IV: 0.2 mg after delivery of anterior shoulder, after delivery of placenta, or during puerperium; may be repeated every 2 to 4 hours as needed. **Note:** IV administration should only be considered during life-threatening situations.

Dosage adjustment for concomitant therapy: Significant drug interactions exist, requiring dose/frequency adjustment or avoidance. Consult drug interactions database for more information.

Dosing: Kidney Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling. However, need for dosage adjustment unlikely as limited excretion in the urine (Ref).

Dosing: Liver Impairment: Adult

There are no dosage adjustments provided in the manufacturer's labeling (has not been studied); use with caution.

Dosing: Older Adult

Refer to adult dosing.

Adverse Reactions

The following adverse drug reactions are derived from product labeling unless otherwise specified.

Frequency not defined:

Cardiovascular: Hypertension, hypotension, palpitations, tachycardia, thrombophlebitis, vasoconstriction, vasospasm

Dermatologic: Diaphoresis, skin rash

Endocrine & metabolic: Water intoxication

Gastrointestinal: Abdominal pain, diarrhea, nausea, unpleasant taste, vomiting

Genitourinary: Hematuria

Nervous system: Dizziness, hallucination, headache, seizure

Neuromuscular & skeletal: Lower limb cramp

Otic: Tinnitus

Respiratory: Dyspnea, nasal congestion

Postmarketing:

Cardiovascular: Acute myocardial infarction (Tsui 2001), angina pectoris, atrioventricular block, bradycardia (Ibrahim 2008), chest pain (Ibrahim 2008), coronary artery vasospasm (Jang 2023), ventricular fibrillation, ventricular tachycardia

Hypersensitivity: Anaphylaxis

Nervous system: Cerebrovascular accident, paresthesia

Contraindications

Hypersensitivity to methylergonovine or any component of the formulation; hypertension; toxemia; pregnancy.

Warnings/Precautions

Disease-related concerns:

- Coronary artery disease: Patients with coronary artery disease (CAD) or risk factors for CAD (eg, diabetes, high cholesterol, obesity, smoking) may be more likely to develop myocardial ischemia and infarction following methylergonovine-induced vasospasm.
- Labor: Use with caution in the second stage of labor.
- Sepsis: Use with caution in patients with sepsis.
- Vascular disease: Use with caution in patients with obliterative vascular disease.

Other warnings/precautions:

- IV administration: Not for routine IV administration due to risk of inducing sudden hypertensive and cerebrovascular accidents. IV administration should only be considered during life-threatening situations.
- Medication errors: Inadvertent administration to newborns has been reported.

Dosage Forms: US

Excipient information presented when available (limited, particularly for generics); consult specific product labeling.

Solution, Injection, as maleate:

Generic: 0.2 mg/mL (1 mL)

Solution, Injection, as maleate [preservative free]:

Generic: 0.2 mg/mL (1 mL)

Tablet, Oral, as maleate:

Methergine: 0.2 mg [contains methylparaben (methyl p-hydroxybenzoate), propylparaben (propyl p-hydroxybenzoate)]

Generic: 0.2 mg

Generic Equivalent Available: US

Yes

Pricing: US

Solution (Methylergonovine Maleate Injection)

0.2 mg/mL (per mL): \$18.00 - \$38.11

Tablets (Methergine Oral)

0.2 mg (per each): \$74.70

Tablets (Methylergonovine Maleate Oral)

0.2 mg (per each): \$67.23 - \$70.97

Disclaimer: A representative AWP (Average Wholesale Price) price or price range is provided as reference price only. A range is provided when more than one manufacturer's AWP price is available and uses the low and high price reported by the manufacturers to determine the range. The pricing data should be used for benchmarking purposes only, and as such should not be used alone to set or adjudicate any prices for reimbursement or purchasing functions or considered to be an exact price for a single product and/or manufacturer. Medi-Span expressly disclaims all warranties of any kind or nature, whether express or implied, and assumes no liability with respect to accuracy of price or price range data published in its solutions. In no event shall Medi-Span be liable for special, indirect, incidental, or consequential damages arising from use of price or price range data. Pricing data is updated monthly.

Administration: Adult

IV: Avoid intra-arterial or periarterial injections. Administer slowly over a period of no less than 60 seconds with careful monitoring of blood pressure. Do not routinely administer IV because of the possibility of inducing sudden hypertension and cerebrovascular accident; only consider IV administration during life-threatening situations.

IM: May be administered intramuscularly.

Oral: Available in tablets for oral administration.

Hazardous Drugs Handling Considerations

Hazardous agent (NIOSH 2024 [table 2]).

Use appropriate precautions for receiving, handling, storage, preparation, dispensing, transporting, administration, and disposal. Follow NIOSH and USP 800 recommendations **and** institution-specific policies/procedures for appropriate containment strategy (NIOSH 2023; NIOSH 2024; USP-NF 2020).

Note: Facilities may perform risk assessment of some hazardous drugs to determine if appropriate for alternative handling and containment strategies (USP-NF 2020). Refer to institution-specific handling policies/procedures.

Storage/Stability

Injection: Store at 2°C to 8°C (36°F to 46°F). Protect from light. The following stability information has also been reported: May be stored at room temperature for up to 14 days (Cohen 2007).

Tablet: Store at 20°C to 25°C (68°F to 77°F); excursion permitted to 15°C to 30°C (59°F to 86°F); protect from light.

Use: Labeled Indications

Postpartum hemorrhage: Management of uterine atony, hemorrhage and subinvolution of the uterus following delivery of the placenta; control of uterine hemorrhage following delivery of the anterior shoulder in the second stage of labor.

Medication Safety Issues

Sound-alike/look-alike issues:

Methergine may be confused with Brethine

Methylergonovine and terbutaline parenteral dosage forms look similar. Due to their contrasting indications, use care when administering these agents.

High alert medication:

The Institute for Safe Medication Practices (ISMP) includes this medication among its list of drugs (contraindicated in pregnancy) which have a heightened risk of causing significant patient harm when used in error (ISMP List of High-Alert Medications in Community/Ambulatory Care Settings ©ISMP 2021).

Administration issues:

Inadvertent administration of methylergonovine to newborns has been reported in place of routine medications (eg, vitamin K or hepatitis B vaccine); store methylergonovine injection separately from medications used for neonates.

Metabolism/Transport Effects

Substrate of CYP3A4 (Major with inhibitors), CYP3A4 (Minor with inducers); **Note:** Assignment of Major/Minor substrate status based on clinically relevant drug interaction potential

Drug Interactions

(For additional information: Launch drug interactions program)

Note: Interacting drugs may **not be individually listed below** if they are part of a group interaction (eg, individual drugs within “CYP3A4 Inducers [Strong]” are NOT listed). For a complete list of drug interactions by individual drug name and detailed management recommendations, use the drug interactions program by clicking on the “Launch drug interactions program” link above.

Alpha-/Beta-Agonists: Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates) may increase vasoconstricting effects of Alpha-/Beta-Agonists. *Risk X: Avoid*

Alpha1-Agonists: Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates) may increase vasoconstricting effects of Alpha1-Agonists. *Risk X: Avoid*

Aprepitant: May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk C: Monitor*

Beta-Blockers: May increase vasoconstricting effects of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk C: Monitor*

Bromocriptine: Ergot Derivatives may increase adverse/toxic effects of Bromocriptine. Management: Use of bromocriptine with other ergot derivatives is not recommended and use of other ergot derivatives should not occur within 6 hours of bromocriptine administration. *Risk D: Consider Therapy Modification*

Chloroprocaine (Systemic): May increase hypertensive effects of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk C: Monitor*

Clofazimine: May increase serum concentrations of CYP3A4 Substrates (High risk with Inhibitors). *Risk C: Monitor*

CYP3A4 Inducers (Strong): May decrease serum concentrations of Methylergonovine. *Risk C: Monitor*

CYP3A4 Inhibitors (Moderate): May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk C: Monitor*

CYP3A4 Inhibitors (Strong): May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk X: Avoid*

CYP3A4 Inhibitors (Weak): May increase serum concentrations of Methylergonovine. *Risk C: Monitor*

Dihydroergotamine: Ergot Derivatives may increase vasoconstricting effects of Dihydroergotamine. *Risk X: Avoid*

Ergot Derivatives: Adverse/toxic effects of Methylergonovine and Ergot Derivatives may increase with coadministration. *Risk C: Monitor*

Erythromycin (Systemic): May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk X: Avoid*

Fosamprenavir: May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk X: Avoid*

Fusidic Acid (Systemic): May increase serum concentrations of CYP3A4 Substrates (High risk with Inhibitors). Management: Consider avoiding this combination if possible. If required, monitor patients closely for increased adverse effects of the CYP3A4 substrate. *Risk D: Consider Therapy Modification*

Grapefruit Juice: May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk C: Monitor*

Itraconazole: May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk X: Avoid*

Lenacapavir: May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). Management: Consider alternatives to the coadministration of lenacapavir and these vasoconstrictive ergot derivatives that are metabolized by CYP3A4 when possible, as product labeling states concomitant use is not recommended. If combined, monitor for ergot toxicity. *Risk D: Consider Therapy Modification*

Letermovir: May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk X: Avoid*

Lisuride: May increase adverse/toxic effects of Ergot Derivatives. *Risk X: Avoid*

Metergoline: Ergot Derivatives may increase adverse/toxic effects of Metergoline. Management: Combined use of metergoline with other ergot alkaloids after birth and during the postpartum period is specifically not recommended. *Risk D: Consider Therapy Modification*

Methysergide: Ergot Derivatives may increase vasoconstricting effects of Methysergide. *Risk X: Avoid*

Nefazodone: May increase serotonergic effects of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). This could result in serotonin syndrome. Nefazodone may increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk X: Avoid*

Nicotine: May increase vasoconstricting effects of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk C: Monitor*

Nitroglycerin: Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates) may decrease vasodilatory effects of Nitroglycerin. Nitroglycerin may increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). Management: Avoid the use of ergot derivatives in patients receiving nitroglycerin for angina (or in any angina patient) if possible. If combined, monitor for decreased effects of nitroglycerin and increased adverse effects of the ergot derivative (eg, ergotism). *Risk D: Consider Therapy Modification*

Pergolide: May increase adverse/toxic effects of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk C: Monitor*

Prostaglandins: Adverse/toxic effects of Methylergonovine and Prostaglandins may increase with coadministration. *Risk C: Monitor*

Reboxetine: May increase hypertensive effects of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk C: Monitor*

Roxithromycin: May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk X: Avoid*

Serotonergic Agents (High Risk): Ergot Derivatives may increase serotonergic effects of Serotonergic Agents (High Risk). This could result in serotonin syndrome. Management: Monitor for signs and symptoms of serotonin syndrome/serotonin toxicity (eg, hyperreflexia, clonus, hyperthermia, diaphoresis, tremor, autonomic instability, mental status changes) when these agents are combined. *Risk C: Monitor*

Serotonin 5-HT_{1D} Receptor Agonists (Triptans): May increase vasoconstricting effects of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk X: Avoid*

Sulprostone: May increase vasoconstricting effects of Ergot Derivatives. *Risk C: Monitor*

Tipranavir: May increase serum concentrations of Ergot Derivatives (Vasoconstrictive CYP3A4 Substrates). *Risk X: Avoid*

Pregnancy Considerations

Methylergonovine is intended for use after delivery of the anterior shoulder, after delivery of the placenta, or during the puerperium; use is contraindicated during pregnancy.

Methylergonovine is used for the treatment of postpartum hemorrhage due to uterine atony. Methylergonovine is recommended when a supplemental uterotonic agent is needed; due to maternal side effects it is not preferred for initial use (ACOG 183 2017).

Breastfeeding Considerations

Methylergonovine is present in human milk.

Data related to the presence of methylergonovine in breast milk are available from multiple studies (Erkkola 1978; Iwamura 1981; Nakamichi 2012; Vogel 2004).

- Methylergonovine 0.125 mg was administered orally 3 times daily for 5 days to 8 women treated for incomplete postpartum involution. Milk concentrations were evaluated 1 and 8 hours after the morning dose on day 5. The highest milk concentrations were observed 1 hour after the dose and ranged from <0.5 ng/mL (lower limit of detection) to 1.3 ng/mL (Erkkola 1978). Using data from this study and a milk concentration of 1.3 ng/mL, the estimated daily infant dose via breast milk is 195 ng/kg/day and the relative infant dose (RID) of methylergonovine is 3.9% compared to the weight-adjusted maternal. Higher doses are recommended in current product labeling.

- In general, breastfeeding is considered acceptable when the RID of a medication is <10% (Anderson 2016; Ito 2000).

Adverse events may be observed in breastfed infants and may include agitation, diarrhea, increased blood pressure, bradycardia, seizures, tachycardia, or vomiting (Methylergonometrine 2014). Prolactin concentrations (and possibly milk supply) may be changed following methylergonovine administration; this may be dependent upon dose and route of administration (Del Pozo 1975; Weiss 1975).

Some manufacturers do not recommend breastfeeding during therapy or for 12 hours after the last dose due to adverse reactions reported in breastfeeding infants. However, other sources note use may be acceptable if breastfeeding (Ito 2000).

Monitoring Parameters

BP, especially with IV administration.

Mechanism of Action

Increases the tone, rate and amplitude of contractions on the smooth muscles of the uterus, producing sustained contractions which shortens the third stage of labor and reduces blood loss.

Pharmacokinetics (Adult Data Unless Noted)

Onset of action: Oxytocic: Oral: 5 to 10 minutes; IM: 2 to 5 minutes; IV: Immediately.

Absorption: Rapid.

Distribution: V_d : IM: 56.1 ± 17 L.

Metabolism: Hepatic.

Bioavailability: Oral: 60%; IM: 78%.

Half-life elimination: IM: 3.39 hours (range: 1.5 to 12.7 hours).

Time to peak, serum: Oral: 1.12 ± 0.82 hours (may be increased to ~3 hours in patients receiving continuous oxytocic agents); IM: 0.41 ± 0.21 hours.

Excretion: Urine and feces.

Patient Counseling Points

(For additional information see "Methylergonovine (methylergometrine): Patient drug information")

What is this drug used for?

- It is used to stop or treat bleeding that happens after a birth. If you have been given this drug for some other reason, talk with your doctor for more information.

All drugs may cause side effects. However, many people have no side effects or only have minor side effects. Call your doctor or get medical help if any of these side effects or any other side effects bother you or do not go away:

- Headache
- Stomach pain
- Upset stomach or throwing up

WARNING/CAUTION: Even though it may be rare, some people may have very bad and sometimes deadly side effects when taking a drug. Tell your doctor or get medical help right away if you have any of the following signs or symptoms that may be related to a very bad side effect:

- High or low blood pressure like very bad headache or dizziness, passing out, or change in eyesight
- Weakness on 1 side of the body, trouble speaking or thinking, change in balance, drooping on one side of the face, or blurred eyesight
- Seizures
- Signs of an allergic reaction, like rash; hives; itching; red, swollen, blistered, or peeling skin with or without fever; wheezing; tightness in the chest or throat; trouble breathing, swallowing, or talking; unusual hoarseness; or swelling of the mouth, face, lips, tongue, or throat.

Note: This is not a comprehensive list of all side effects. Talk to your doctor if you have questions.

Consumer Information Use and Disclaimer: This information should not be used to decide whether or not to take this medicine or any other medicine. Only the healthcare provider has the knowledge and training to decide which medicines are right for a specific patient. This information does not endorse any medicine as safe, effective, or approved for treating any patient or health condition. This is only a limited summary of general information about the medicine's uses from the patient education leaflet and is not intended to be comprehensive. This limited summary does NOT include all information available about the possible uses, directions, warnings, precautions, interactions, adverse effects, or risks that may apply to this medicine. This information is not intended to provide medical advice, diagnosis or treatment and does not replace information you receive from

the healthcare provider. For a more detailed summary of information about the risks and benefits of using this medicine, please speak with your healthcare provider and review the entire patient education leaflet.

Brand Names: International

International Brand Names by Country

For country code abbreviations (show table)

- (AE) United Arab Emirates: Methergin;
- (AR) Argentina: Basofortina;
- (AT) Austria: Methergin;
- (BD) Bangladesh: Methergin | Metherspan;
- (BF) Burkina Faso: Lerin;
- (BG) Bulgaria: Methergin | Methylergobrevin | Methylergometrin;
- (BR) Brazil: Ergometrin | Methergin;
- (CH) Switzerland: Methergin;
- (CI) Côte d'Ivoire: Lerin;
- (CL) Chile: Methergin;
- (CO) Colombia: Methergin;
- (DE) Germany: Methergin;
- (DO) Dominican Republic: Methergin;
- (EC) Ecuador: Methergina;
- (EG) Egypt: Methergin;
- (ES) Spain: Methergin;
- (FI) Finland: Methergin | Myomergin;
- (FR) France: Methergin;
- (GR) Greece: Demergin | Ergovit | Methergine | Mitrotan;
- (HK) Hong Kong: Methergin;
- (ID) Indonesia: Methergin | Metherinal | Methovin | Methyl ergometrine | Methylergometrine | Metiagin | Metilat | Metvell | Myotonic | Pospargin;
- (IL) Israel: Methergin;
- (IN) India: Algometrine | Egrochem | Ergagin | Ergogin | Ergolin | Ergoriv | Gotrin | Hicon | Ingagen-m | Lerin | Limitex | Mem | Mem mac | Memcad | Memjet | Mergox | Merzen | Metergo | Metermin | Metharmed | Methastar | Methergin | Methygin | Methylergometrine | Utergin | Wergomet;
- (IT) Italy: Methergin;
- (JO) Jordan: Methergin;
- (JP) Japan: Derganin | Derganin taiyo | Levospan | Metenarin | Methecrine | Methergin | Methyl-egyl kayaku | Methylergometrine | Partan m | Ryegonovin | Takimetrim m | Uruteon;
- (KR) Korea, Republic of: Ergospar m | Methergin | Unidergin | Unidergine;
- (KW) Kuwait: Methergin;
- (LB) Lebanon: Methergine;
- (LT) Lithuania: Methergin;
- (LV) Latvia: Methergin | Methylergometrin;
- (MA) Morocco: Methergin;
- (MX) Mexico: Methergin;
- (MY) Malaysia: Methergin;
- (NO) Norway: Methergin;
- (PE) Peru: Methergin | Neometrin;
- (PH) Philippines: Ergojen | Ergon | Ergotec | Mergot | Methergin | Mexytrine | Usa-methylergometrine maleate | Usamema | Utergin;
- (PK) Pakistan: Ergomin | Methergin;
- (PR) Puerto Rico: Methergine | Methylergonovine maleate;
- (PT) Portugal: Methergin | Metilergometrina;
- (PY) Paraguay: Basofortina | Femitranol;

- (QA) Qatar: Methergin | Metiler;
- (SA) Saudi Arabia: Methergin;
- (SE) Sweden: Methergin;
- (SR) Suriname: Ergogin | Methyl ergometrine;
- (TH) Thailand: Ermetrin | Gynogin | Methergin | Methylergometrine;
- (TN) Tunisia: Ergotyl;
- (TR) Turkey: Methergin | Metiler | Uterjin;
- (TW) Taiwan: Methergin | Methylergonovine;
- (UA) Ukraine: Methylergometrin;
- (UG) Uganda: Ergomin;
- (UY) Uruguay: Methergin;
- (VE) Venezuela, Bolivarian Republic of: Methergin;
- (ZM) Zambia: Ergogin

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